

Finasteride in Androgenetic Alopecia

Efficacy and Tolerance of Finasteride

An extension of the above study to 5 years showed that finasteride 1 mg/day was well tolerated and led to durable improvements in scalp hair growth.

Finasteride is generally well tolerated; side effects are typically mild and rarely require discontinuation of therapy. Rare side effects may include decreased libido and erectile dysfunction. At present, there is no proven benefit for finasteride in women based on Western studies. A placebo-controlled trial in post-menopausal women with androgenetic alopecia (AGA) using finasteride 1 mg/day over 1 year showed no significant improvement. However, a study using finasteride 5 mg/day in Asian women suggested that it may be an effective and safe treatment for normoandrogenic women with female pattern hair loss (FPHL). Further research is needed to confirm the efficacy of finasteride in women.

Finasteride in Asian Populations

Finasteride is effective in Asian patients with AGA. In Taiwanese men, treatment with finasteride 1 mg/day for 1 year slowed hair loss progression and increased hair growth. A clinical study by Kawashima et al. found that 58%, 54%, and 6% of Japanese men with male pattern hair loss (MPHL) receiving finasteride 1 mg, finasteride 0.2 mg, and placebo, respectively, showed improvement based on global photographic assessment. All efficacy measures were numerically better with the 1 mg dose compared to the 0.2 mg dose, supporting the use of finasteride 1 mg/day in Japanese men. Additionally, a recent Japanese study involving 3,177 AGA patients demonstrated that long-term oral finasteride use maintained progressive hair regrowth without significant adverse effects.

Despite these findings, no published comparative studies exist between Asian and Caucasian patients. Interestingly, while oral finasteride improved quality of

life in Japanese AGA patients, it did not reduce their anxiety related to hair loss.

Treatment Options for Women

(1) Topical Minoxidil Solution

As in men, the standard regimen for women is 1 mL of topical minoxidil solution applied twice daily. However, data in women are less conclusive. In a randomized, placebo-controlled trial involving Japanese women with AGA, Tsuboi et al. reported moderate or greater improvement in 29.2% of patients using the 1% minoxidil solution compared to 11.8% in the placebo group.

Two studies comparing the 5% and 2% minoxidil solutions in 493 women found no significant difference in efficacy based on hair count. A recent comparative study in Caucasians between 5% minoxidil foam and 2% solution also showed similar outcomes.

Patients should be informed about the possibility of an initial, transient telogen effluvium, which should not prompt discontinuation of treatment. Side effects include hypertrichosis, occurring in 6% of women using the 2% solution and 14% of those using the 5% solution. Hypertrichosis typically affects the face and resolves within 1–6 months after stopping treatment. In Caucasian populations, it often diminishes or disappears after approximately 1 year, even with continued minoxidil use.

(2) Oral Anti-Androgens

Cyproterone acetate, spironolactone, and flutamide may be used as alternatives to minoxidil, particularly in women who do not respond to or cannot tolerate topical therapy. However, most anti-androgen treatments have not been rigorously evaluated in FPHL. Better responses are generally observed in women with hyperandrogenism.

Side effects are more common with cyproterone acetate and spironolactone. Although some small studies have investigated the efficacy of anti-androgens in

AGA, this approach is not routinely recommended due to limited evidence and potential adverse effects.